MEDITECH

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Centers for Medicare & Medicaid Services
Department of Health and Human Services
P.O. Box 8013
Baltimore, MD 21244-8013

Attention: CMS-0042-NC

Dear Administrator Oz,

On behalf of Medical Information Technology, Inc., I am pleased to offer comments on the *Request for Information; Health Technology Ecosystem.*

MEDITECH empowers healthcare organizations to expand their vision of what's possible with Expanse, the intelligent EHR platform. Around the globe, Expanse transforms care and ushers organizations into the future with AI-infused solutions, personalized workflows, next-level interoperability, and predictive analytics — all working together to drive better outcomes.

Please kindly find our responses to the questions noted within the RFI as you consider the development of future regulations.

Technology Vendors, Data Providers, and Networks

1. Ecosystem

TD-1: What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

MEDITECH echoes comment by the EHRA:

"In the short term, CMS can stimulate developer interest by promoting the adoption of standardized APIs that enable seamless integration with Medicare data systems. Streamlined and transparent regulatory requirements would also lower barriers to entry for new digital health products targeting beneficiaries and caregivers.

Longer term, CMS should implement reimbursement models that incorporate digital health products into care delivery and quality programs. This includes end-to-end alignment, establishing certification requirements and exchange standards for all involved parties (not just EHR developers and providers), along with downstream incentives for consistent use by providers, payers, public health agencies, and others."

2. Digital Identity

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

Digital identity credentials bolster security and trust, facilitating consistent identity verification across diverse systems, which can enhance interoperability and patient safety.

However, scaling digital identity implementation poses considerable workflow and integration challenges, especially for organizations managing identity across both internal systems and external applications. A primary obstacle is establishing cross-organizational trust, specifically how third-party applications and healthcare providers authenticate and accept credentials without established relationships. The lack of defined governance, implementation standards, and use cases may hinder broad adoption.

3. Technical Standards and Certification

TD-4: How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can foster greater interoperability by mandating the use of standards across all parties involved in data exchange. This means requiring standards not only for healthcare providers and EHR developers but also for payers, public health agencies, and other stakeholders. Rallying the entire community around a single certification, like the Da Vinci standard, and incorporating feedback from EHR vendors will help achieve this. When standards are only required in one direction, it creates an imbalance that limits true interoperability and discourages investment in scalable solutions.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

A nationwide provider directory of FHIR endpoints would greatly facilitate data exchange. However, for this to move forward, data would need to be easily discoverable and managed appropriately.

MEDITECH does not support shifting the cost burden to users. Rather, it should be seen as foundational infrastructure and supported through federal investment.

TD-6. What unique interoperability functions does TEFCA perform?

MEDITECH echoes comment by the EHRA:

"TEFCA's unique value lies in its potential to standardize and scale nationwide health information exchange by establishing a clear governance framework, defined use cases, and consistent technical requirements. The EHR Association has consistently supported TEFCA's goals of reducing fragmentation in exchange models and expanding participation across a broader range of stakeholders, including providers, payers, public health agencies, and health IT developers.

One of TEFCA's distinguishing features is that it encourages adoption of existing standards (including HL7 FHIR and IHE protocols), helping organizations communicate using uniform data structures and reducing the burden of managing proprietary or non-standard connections.

Importantly, TEFCA offers a centralized alternative to costly, siloed HIE participation, which may improve cost efficiency for providers, especially those currently paying to connect with multiple regional networks. If TEFCA can truly deliver on scalable, standards-based exchange, it could reduce duplicative infrastructure investments across the ecosystem.

The EHR Association views TEFCA as a promising complement, not a replacement, for other forms of interoperability, and continues to encourage CMS and ASTP/ONC to ensure its implementation is inclusive, technically feasible, and driven by mature standards."

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

The United States Core Data for Interoperability (USCDI) has significantly advanced healthcare data exchange by effectively identifying foundational data classes and elements, leading to improved interoperability. This effort has fostered better collaboration between vendors and clients, resulting in a repeatable, structured and standardized process with enhanced consistency.

Noted limitations of USCDI include gaps in meaningful, well-vetted use cases and data definitions. Further, electronic health records vendors are the only stakeholder required to establish and maintain certification while those on the other side of exchange (e.g. payers, app vendors, public health) do not. This leads to an imbalance of standards and creates barriers to true interoperability. To address these limitations, we recommend expanding accountability to other exchange actors as well as ensuring data elements are accompanied with high priority use cases and thorough data definitions.

a. Does it contain the full extent of data elements you need?
b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?
c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

Most of the common data have been profiled and a core data set is largely established. As USCDI's scope expands beyond a foundational common dataset, data will branch out to be more relevant to specialized use cases, which may cause undue burden. MEDITECH encourages more USCDI+ data sets rather than adding to USCDI. Consideration should be given to high priority use cases with the largest stakeholder impact to encourage buy in.

d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

MEDITECH strongly recommends that data remain well-defined and structured. Standardized, structured data, such as that facilitated by FHIR profiles and consistent value sets, are essential for ensuring data integrity, promoting true interoperability, and building reliable, scalable healthcare applications.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The U.S. Core Data for Interoperability (USCDI) and the underlying principles that enable effective Clinical Decision Support (CDS) have significantly advanced healthcare data standardization and communication. These initiatives have provided a foundational framework, demonstrating the clear value of adopting shared standards across the healthcare ecosystem. However, several years into their implementation, the benefits of standardization appear to be disproportionately realized.

To truly unlock the full potential of interoperability, it is imperative we move beyond a one-sided adoption model. MEDITECH recommends actively encouraging and incentivizing payers and Public Health Agencies (PHAs) to adhere to the same data standards that have been established for other stakeholders. This current disparity creates significant challenges, as evidenced by the difficulties encountered with electronic prior authorization (ePA), where Electronic Health Record (EHR) vendors struggle to find payers willing to test due to a lack of consistent standard implementation. This lack of standardized data also directly hampers the effectiveness of Clinical Decision Support tools, as they rely on comprehensive and consistent information from all sources to provide accurate and timely guidance to clinicians.

TD-9. Regarding certification of health IT:

a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

Redefining health IT certification to prioritize API-enabled capabilities over traditional software functionality offers benefits. This shift allows for access to the entire patient record, moving beyond visit-based data (like C-CDA) to create a comprehensive,

longitudinal view of a patient's health. Furthermore, certification based on APIs ensures systems can precisely meet and support diverse, specific use case requirements. Amidst this evolution, the USCDI remains important as the standardized base foundation, providing the common language for data exchange. This pivotal shift necessitates the use of well-vetted testing pools that can simulate complex, real-world, multi-system interactions, as such rigorous testing is vital for ensuring true interoperability, identifying potential vulnerabilities, building essential trust among stakeholders, and driving continuous improvement in the realm of health IT.

b. What would be the drawbacks?

A significant drawback is the potential for new certification requirements to inadvertently burden healthcare sites. Consideration should include the practical side for implementers as prioritizing new API capabilities could create major hurdles, including overlooking ease of adoption, necessary infrastructure changes, or impact to existing workflows. Our goal must be to ensure that the benefits of enhanced interoperability—like comprehensive patient records and efficient data exchange—actually lighten the administrative load and improve patient care at the front lines, rather than piling on new layers of complexity and cost.

c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

ASTP/ONC could revise health IT certification criteria by focusing on fewer, more impactful requirements that are available for a larger stakeholder pool. The emphasis should be on practical solutions that enable broader data exchange while acknowledging the inherent complexities of unstructured information.

d. What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?

While API data requests and responses offer significant benefits, widespread provider adoption hinges on substantial incentives, as implementation demands considerable staff effort and training. Although Information Blocking rules establish compliance, providers currently receive minimal practical benefits for delivering robust API responses. To address this, CMS could enhance incentive alignment, such as rewarding automated electronic case reporting (eCR) in lieu of manual submissions. Ultimately, policy strategies should be collaboratively developed with providers, who are best equipped to advise on how API usage can be integrated into clinical workflows while considering resource limitations.

e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Bulk data transfer functionality lacks thorough testing and validation due to limited utilization. Further, this functionality can be quite taxing on systems. To demonstrate benefit, a robust and scalable infrastructure is essential which must facilitate an efficient data transfer process that places minimal strain on systems and the network. This approach prevents disruptions to clinical workflows and avoids imposing prohibitive costs on healthcare providers.

Concerns include timely access to performance data as well as provider attribution. Providers need consistent, usable information to track their ongoing quality performance and bulk transfer handles periodic reporting. EHRs must be capable of generating and presenting real-time or near real-time performance to allow for timely quality improvement efforts to occur. Further, accurately attributing quality performance to individual providers or groups, especially in complex care settings with multiple contributing clinicians, can be challenging even with comprehensive data. Bulk transfer needs to support sophisticated attribution rules to avoid misrepresentation of performance.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

MEDITECH requests clarification on the scope and definition of "all data elements". "All data elements" inherently introduces complexity particularly with legacy data which is largely unstructured. Greater specificity will help ensure alignment across technology developers and stakeholders and avoid misaligned implementation expectations.

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a. Should this capability be revised to specify standardized API requirements for EHI export?

MEDITECH echoes comment by the EHRA:

> "Yes, the EHI Export capability should be revised to include standardized API requirements. In its current form, EHI Export has not met its intended goals of simplifying patient access to their complete electronic health information or streamlining provider transitions between health IT systems. There are numerous variables contributing to this, but chief amongst them is the lack of defined, commonly accepted standards for both data format and delivery mechanisms. While developers are required to publicly document their export specifications, healthcare data remains highly complex and difficult to interpret consistently without a shared, well-understood standard, regardless of how detailed and thorough a developer's format specification documentation is.
>
> This is especially true for population-level exports, where the volume and variability of data make it unrealistic to assume a seamless, low-burden export and import process. Even with thorough documentation, significant human intervention is typically required to process such exports meaningfully.
>
> The EHR Association recommends re-scoping the EHI Export criterion to focus not on "all EHI" but instead on all USCDI data and reference guidance on how to support EHI Export using FHIR Bulk Data that includes an organized USCDI data set. We suggest that EHI Export be repurposed considering these realities and scope to a HL7 FHIR-based data set that can be shared using HL7 FHIR Bulk Data Access for a single or multiple patients that contains all USCDI scoped data as expressed in HL7 FHIR US Core, and that can grow not only based on HL7 FHIR US Core expansion (which is only a limited data set) but on an identified combination of HL7 FHIR US Core and profiles from implementation guides reflecting specialized data such as full laboratory results, care plans, and other full data sets. Any data not available in FHIR would inherently require more effort by the receiver and should not be expected to flow seamlessly under a standardized export.
>
> Additionally, it should be clarified that EHI Export is not appropriate as a standalone solution for HIT-to-HIT migration, which often requires tailored workflows and complex mappings beyond a universal dataset. In effect, EHI Export should be repositioned as a FHIR Bulk Data use case with a defined implementation guide, rather than an open-ended export of all possible EHI."

b. Are there specific workflow aspects that could be improved?

Exporting Electronic Health Information (EHI) presents challenges unrelated to workflow execution. Developers should maintain discretion over initiating and conducting exports, aligning with individual product design and usage. Standardizing workflow specifics across vendors would introduce needless inflexibility and complication without providing tangible benefits. Certification should prioritize demonstrating capability and reliable functionality of the export feature, rather than specifying internal processes or user interfaces.

c. Should CMS consider policy changes to support this capability's use?

Requiring EHI Export is unnecessary due to its infrequent usage, primarily triggered by patient data requests or provider EHR transitions. Existing standardized exchange mechanisms, like FHIR and C-CDA, already effectively address most real-world interoperability needs. Thus, mandating EHI Export adds no value.

Instead, promoting the widespread adoption of standard exchanges such as HL7 FHIR and CDA would better serve the majority of interoperability requirements. For the few situations demanding complete datasets, custom methods can be used until USCDI or FHIR profiles evolve to handle all EHI use cases.

4. Data Exchange

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

APIs providing access to the entirety of a patient's electronic health information (EHI) can open opportunities such as population level research.

a. What are the primary obstacles to this?

Primary obstacles include data volume, unstructured data, and performance issues when accessing multiple patients. The entire EHI can be vast and largely unstructured which poses substantial challenges for systems to be performant and scalable. APIs must be robust enough to query information without degrading system responsiveness for clinical users, and the underlying infrastructure must be capable of processing and serving this data efficiently.

Further, providing direct API access to raw EHI, especially unstructured or decontextualized discrete data, carries the risk of misinterpretation by downstream applications or researchers. Without proper clinical context or data governance frameworks, isolated data points could lead to flawed conclusions or inappropriate clinical decisions.

b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The primary tradeoff between USCDI and full EHI stems from a fundamental gap in standards. USCDI mandates specific data elements and references standards for their representation. Full EHI offers the complete picture, but its lack of universal standardization beyond the USCDI scope presents a significant hurdle for automated processing and trusted exchange. The ongoing challenge is to expand USCDI to encompass more EHI while simultaneously developing robust, standardized methods for accessing and interpreting the remaining unstructured and less standardized data within the broader EHI landscape, all while keeping costs and provider burden in mind.

TD-14. Regarding networks' use of FHIR APIs:

a. How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?

MEDITECH offers Traverse Exchange, an advanced nationwide health information exchange network for US customers that enables participating MEDITECH customers to share health data seamlessly and securely with one another as well as with organizations using other vendor EHRs and exchange networks that comply with interoperability standards. Currently, it is searchable by patient only, not organization ID or NPI.

b. How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?

These connections are established using *FHIR (g)(10) endpoints.*

c. Do you interconnect with other networks? Under what frameworks (for example, TEFCA, private agreements)?

The solution positions MEDITECH and its customers well for TEFCA, extending QHIN services through its network via connectivity to the Health Gorilla QHIN. The goal is for all MEDITECH customers to be connected via Traverse Exchange.

TD-15. Regarding bulk FHIR APIs:

a. How would increased use of bulk FHIR improve use cases and data flow?

MEDITECH echoes comment by the EHRA:

"Increased use of Bulk FHIR has the potential to significantly improve data flow and enable advanced use cases such as digital quality measurement, population health analytics, and more efficient payer-provider data exchange. It provides a standardized, scalable approach for extracting large datasets, particularly useful when aggregating information across patient populations. However, realizing this promise depends on greater adoption, maturity of implementation guides, and availability of robust testing tools. As noted in our comments on the FY2026 IPPS proposed rule, EHRs are certified to support Bulk FHIR; however, the limited deployment by providers and other stakeholders has hindered real-world experience. To bridge this gap, CMS could support targeted incentives and collaborative pilot programs that enable providers and developers to implement Bulk FHIR in specific, meaningful use cases—ultimately demonstrating its value and building industry confidence.

For example, HRSA's UDS+ initiative aimed to receive data from any FHIR-enabled system—including EHRs and specialty systems like dental software. While promising in theory, the reality proved more complex. HRSA's requirement for de-identified data conflicted with its expectation to receive multiple data submissions and reconcile them into a unified patient-level view. Without identifiers, accurate deduplication was impossible. The burden of aggregation was ultimately placed back on the submitting provider. This example highlights how the benefits of FHIR can be lost without aligned infrastructure, standardized deduplication strategies, and consistent support across health IT systems."

b. What are the potential disadvantages of their use?

The conceptualization and specifications for bulk FHIR APIs are functionally sound and capable of facilitating data exchange as outlined. However, most organizations frequently lack the processing capacity for data volume, processing, and computational power essential for effective utilization at the necessary scale. While subsequent modifications aimed at reducing data volume may offer some mitigation, the absence of substantial investment and funding for the establishment of technical infrastructure and databases renders the practical implementation of these APIs infeasible.

TD-16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

In point-to-point models, security is more readily trusted and controlled. Conversely, within a shared network infrastructure, the identity of access requestors is often not truly known. However, a shared network infrastructure facilitates data exchange between disparate organizations even in the absence of pre-existing relationships.

a. Do current rules encourage scalable network participation?

Current rules and initiatives, particularly TEFCA and its strong emphasis on standards-based FHIR APIs, are strategically designed to encourage and enable scalable network participation. However, the voluntary nature of TEFCA and the practical challenges related to governance complexity, upfront investment, and technical readiness mean that significant ongoing effort is required to fully achieve broad, scalable network participation across the entire healthcare ecosystem.

b. What changes would improve alignment (for example, API unification, reciprocal access)?

A significant change that would improve alignment is to separate document requirements from API data exchange requirements as these should not be the same rules. Trying to force API data into document-centric rules creates unnecessary complexity and is a large barrier. Policies and regulations should clearly differentiate between requirements for document exchange and those for granular API-based data exchange.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

Adequate funding for data sharing development by CMS and/or ASTP/ONC is critical. Presently, funding largely relies on incentives to payers and providers, who in turn fund EHRs and data sharing networks. This indirect funding model restricts development work beyond regulatory mandates. Direct innovation incentives for data sharing networks or EHR vendors would guarantee sustainability.

5. Compliance

TD-19. Regarding price transparency implementation:

a. What are current shortcomings in content, format, delivery, and timeliness?

Current limitations in price transparency implementation manifest across content, format, delivery, and timeliness. A primary shortcoming is the extensive volume of data that hospitals are mandated to compile into machine-readable files, encompassing all items and services.

Furthermore, hospitals confront considerable challenges stemming from the substantial data volume originating from disparate internal sources (e.g., chargemasters, billing systems, contract repositories). This fragmented data landscape renders timely compilation difficult. Consequently, comprehensive adherence to the evolving price transparency regulations becomes problematic, frequently resulting in increased auditing activities and associated compliance pressures on healthcare organizations.

b. Which workflows would benefit most from functional price transparency?

Workflow optimization opportunities for enhanced price transparency encompass streamlined appointment scheduling, mitigation of surprise billing, facilitation of patient service comparison, and more efficient revenue cycle management processes.

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

Improvements of value would encompass the simplification of price transparency, as the intricacy of current detailed requirements can cause ambiguity. Streamlined requirements would be more conducive to consumers, impose a reduced burden upon healthcare providers, and afford a clearer understanding of aggregate costs. Requirements must be stated clearly and concisely, bundled costs to provide a more transparent framework for cost estimation, and transferring a greater proportion of the onus to payers through the

provision of advanced explanations of benefits, which would consequently alleviate the burden on providers and mitigate the incidence of inaccurate estimates.

d. What would further motivate solution development?

In order to incentivize the development of solutions, the Centers for Medicare & Medicaid Services (CMS) should streamline reporting requirements, as the current volume of data is excessive. Furthermore, a reduction in the frequency of cost adjustments by payers and CMS would mitigate the challenges associated with maintaining precise price lists.

Value-based Care Organizations

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

To encourage adoption and effective use of digital health and care navigation products within ACOs for Medicare beneficiaries, a key strategy involves streamlining performance measures. By focusing on a universal set of outcome-based quality measures that directly align with the capabilities of digital health tools, CMS can more effectively incentivize their use. This approach can reduce the reporting burden on ACOs, simplify demonstration of value, and ties digital health investment directly to measurable improvements in patient outcomes and reductions in health disparities.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

To enhance the integration of critical technologies and themes, such as artificial intelligence (AI), population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement, within Alternative Payment Model (APM) requirements, federal programs and guidelines should be strategically leveraged. Existing frameworks like the CMS SAFER Guides already encompass foundational elements for effective technology utilization, including usability and data integrity. APM requirements could explicitly reference and incentivize adherence to these guides, potentially by linking compliance to favorable shared savings rates or reduced downside risk, thereby fostering a culture of safety and robust technological deployment.

Furthermore, the Quality Payment Program (QPP) Improvement Activities (IAs) category presents a valuable existing mechanism. APM requirements could strengthen this by assigning greater weight or providing additional incentives for specific IAs that demonstrate the robust implementation and effective utilization of AI for advanced risk stratification, comprehensive population health analytics, and digital patient engagement tools. This integration should evolve beyond mere attestation to require outcome-based evidence derived from these implementations, such as demonstrable improvements in patient engagement metrics or reductions in readmissions directly attributable to the use of these advanced technologies.

VB-3. What are essential health IT capabilities for value-based care arrangements? a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.

Essential Health IT capabilities for value-based care arrangements include accurate patient/provider attribution and incorporating analytics that allow for real-time or near real-time provider feedback on quality performance trends. The ability to clearly link patients to the providers primarily responsible for their care, which is fundamental for assigning financial and quality accountability within APMs and for enabling targeted care management interventions which is vital to value-based care. Providers need immediate, actionable insights on their performance against quality measures to course-correct in a timely manner, rather than waiting for retrospective reports. This allows for continuous improvement cycles.

b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

As CMS advances their digital quality measure roadmap, having a timely feedback loop for Digital Quality measures is imperative to identify areas for improvement and guide care quality. This feedback process is a current function of EHRs allowing timely access to quality metrics and trends that can drive continuous improvement and healthcare quality.

MEDITECH also echoes comment by the EHRA:

"One often overlooked capability is bidirectional feedback between CMS and provider systems, especially for quality performance data. As EHRs shift from being the calculation engine to the data source in dQM models, providers still expect real-time visibility into how they're performing on quality measures. Establishing standardized APIs to display performance feedback from CMS or third-party aggregators within the EHR dashboard will be critical.

Pilots like HRSA's UDS+ and CMS's Enhancing Oncology Model demonstrate the promise of FHIR-based digital quality measurement. However, they also reveal practical gaps, such as

difficulty with patient attribution and deduplication across systems that do not uniformly support FHIR. These lessons reinforce the need for health IT capabilities that address real-world implementation challenges, especially where multiple systems must coordinate care for shared patients.

We also recommend CMS and ONC actively solicit provider feedback to identify which capabilities they view as most valuable and actionable. Developer capabilities are only impactful when aligned with provider priorities, workflows, and the incentives that influence their behavior."

VB-4. What are the essential data types needed for successful participation in value-based care arrangements?

Successful participation in value-based care (VBC) arrangements hinges on access to and effective use of a comprehensive array of data types. A critical challenge lies in ensuring consistency in data types and definitions across CMS and various payer programs to enable seamless analysis and accurate reporting. Without this uniformity, aggregating and comparing data for accurate quality measurement, risk stratification, and shared savings calculations becomes an arduous, error-prone, and often impossible task, undermining the very foundation of value-based care.

2. Compliance and Certification

VB-5. In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

The ONC Health IT Certification Program's current certification criteria and standards have been essential in fostering interoperability and standardizing practices for certified health IT developers. This has led to more consistent and dependable data exchange, crucial for value-based care delivery. Nevertheless, the advantages of certification are restricted when other stakeholders, including payers, app developers, and public health agencies, are not subject to comparable standardized requirements. A fully accountable ecosystem with shared data standards, exchange protocols, and certification expectations is vital for the success of value-based care. The absence of broader alignment places a disproportionate burden on providers and their vendors, who must deal with varying data quality and interfaces across external systems.

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

Technology requirements for APMs can reduce complexity while preserving necessary flexibility by achieving consistency in standards, implementation guides (IGs) and value sets. When health IT systems are built to consistently capture and exchange data using agreed-upon standards (e.g., specific FHIR resources and profiles), complexity is drastically cut, allowing data to flow more seamlessly.

Thank you for your time and consideration. We look forward to your responses.